SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Nivolumab · every 2 weeks · previously treated NSCLC

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). · NCCP 00483 · version 13a · 14-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
Aim to control advanced lung cancer, relieve symptoms and prolong life where possible.

**Schedule**
Nivolumab 240 mg IV over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Nivolumab**
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion or injection reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose

Discuss the aim for your exact lung setting, likely benefit, alternatives, important immune risks and how treatment may be paused or stopped. Some side effects need long-term medicines. Ask about fertility, pregnancy, breastfeeding and effects on daily life.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00483 v13a.

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